NCCP 00535 · clinician draft

Avelumab — metastatic Merkel-cell carcinoma

> **The treatment this guide covers** This is the Merkel-cell cancer context after chemotherapy under current NCCP. It is not avelumab maintenance for urothelial cancer. Evidence from untreated first-line Merkel-cell cohorts must not be presented as the evidence for this specific post-chemotherapy indication.
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> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Avelumab — metastatic Merkel-cell carcinoma · support plan

Active medicines: Avelumab

Emetogenic risk: Minimal per NCCP 00535 v7

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Avelumab has minimal emetogenic risk in current NCCP. Premedication for the first four infusions prevents or reduces infusion reactions and is distinct from anti-sickness treatment. Confirm the medicine names and timing on the appointment plan. Other symptom treatments depend on the individual; immune-related symptoms need assessment rather than self-treatment alone.
- Blood count, kidney/liver tests, glucose and thyroid tests before every cycle. The nurse watches for infusion reactions and the team checks for bowel, breathing, skin, endocrine and other immune symptoms. Additional tests depend on symptoms, not just the routine blood panel.
- Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy. During the infusion, tell the nurse immediately about chills, flushing, itching, chest or back pain, dizziness or breathing difficulty.
- Use effective contraception during treatment and for at least one month after the last avelumab dose. Discuss fertility and breastfeeding with the team before treatment; do not substitute the different interval used for another immunotherapy.
- This is the Merkel-cell cancer context after chemotherapy under current NCCP. It is not avelumab maintenance for urothelial cancer. Evidence from untreated first-line Merkel-cell cohorts must not be presented as the evidence for this specific post-chemotherapy indication.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00535 v7 · NCCP 00535 v7 supportive-care section; current product information for medicine-specific precautions

Sources checked 2026-11-06 · SACTCheck v0.84.42 · Regimen-level drafts; prescriber and oncology-pharmacy verification required